



February 23, 2026

The Honorable Robert F. Kennedy, Jr.
Secretary
U.S. Department of Health and Human Services
Hubert H. Humphrey Building
200 Independence Avenue, SW
Washington, DC 20201

RE: RIN 0955-AA13 — Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Secretary Kennedy,

The American Academy of Nursing (Academy) is pleased to offer the following comments in response to the December 23, 2025 request for information regarding actions the Department of Health and Human Services (HHS) can take to accelerate the adoption and use of AI as part of clinical care.¹ With more than 3,200 Fellows, the Academy—an honorific society and policy organization—represents nursing’s most accomplished leaders in policy, research, administration, practice, and academia. Collectively with our Fellows and partners, the Academy aims to create solutions that matter, inspire change that propels transformation, and envision a healthier future.

The Academy appreciates HHS’s commitment to advancing the transformative potential of AI. The Academy underscores that the rapid and expansive growth in AI technology presents a remarkable opportunity for advancing innovations in person-centered care and health care delivery. As AI rapidly impacts health care delivery, health sciences education, and research, it is essential to strengthen and continue developing strong policies that safeguard patients, families, and communities, govern privacy, and uphold data protection. As HHS considers next steps, the Academy offers the following comments in alignment with the questions HHS has outlined.

Question 2: What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

The Academy has long called for APRNs (Advanced Practice Registered Nurses)² to be allowed to practice to the top of their education, certification, and training.³ To promote greater value in clinical care through the use of AI and incentivize its effective use, we encourage HHS to ensure that APRNs are appropriately reimbursed for the expert care they provide using new technologies. For decades, APRNs have experienced arbitrary regulatory limits at the federal level that impacts the length of time for a patient to receive services or medical devices. Medicare and Medicaid conditions of participation restrict

APRN practice by requiring, for example, that physicians supervise anesthesia services provided by certified registered nurse anesthetists (CRNAs) and that every Medicare or Medicaid patient admitted to a hospital be under the care of a physician. Some conditions of coverage restrict APRNs from ordering certain Medicare covered services for patients. These restrictions impact APRNs' ability to practice to the top of their education, certification, and training and unnecessarily burden patient access to care.

Further, we know that when APRNs can practice to the full extent of their education and training, it increases access to care. For example, during the pandemic, many states temporarily lifted the need for APRNs to have collaborative agreements with physicians. This greatly impacted access to care, particularly in rural and underserved areas.⁴

It is also important to note the importance of regular review and updates to regulatory, payment policy, and programmatic design structures at HHS given the exponential growth of AI and rapid pace of change in these landscapes. AI-enabled tools like clinical decision support, remote monitoring, and virtual triage could significantly expand APRN capabilities in medically underserved areas, but only if reimbursement and scope of practice policies keep pace. We encourage HHS to implement guidelines for consistent, regular review and updates to these structures.

As examples, HHS could lead policy to expand payment for AI-supported asynchronous care by APRNs (at 42 CFR § 410.78); add CMS regulatory clarification that AI-assisted documentation, summarization, and risk stratification support, but do not replace, individual APRN practice (at 42 CFR § 410.75); explicitly encourage and recognize AI-enabled care management and early detection when they improve Accountable Care Organization (ACO) quality and utilization patterns; and strengthen state contract expectations so plans can adopt AI-supported APRN interventions (e.g., early deterioration detection, falls risk escalation, medication adherence risk) as recognized Quality Assurance and Performance Improvement (QAPI) activities and performance improvement projects (at 42 CFR § 438.330).

Question 3: For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

As AI increasingly becomes embedded in health care, the need for transparency in its operations becomes more critical. Trust in AI tools depends not only on their accuracy and effectiveness but also on the availability of clear, understandable explanations for AI supplemented clinical decisions. All AI tools used in patient care should show clear, understandable input data sources and if available, explanations should be provided when desired by a consumer. Moreover, patients, families, and clinicians have the right to know when and how AI is being used in diagnoses, treatment, and care delivery. Above all, privacy must be protected.

The use of AI in health care and research brings new potential for security threats and data exposure, especially with non-medical devices. Seemingly non-sensitive data that non-medical devices track could inadvertently expose patients to cyber-fraud or large-scale data breaches impacting their privacy, as well as economic health and well-being. Privacy of patient data could be at risk of breach, for example, if patient records are inappropriately used as a source of data. It is critical that foundational pillars underpinning data privacy and cybersecurity such as the Health Insurance Portability and Accountability Act (HIPAA) be maintained and regularly evaluated for necessary updates. Other statutory areas to examine closely to ensure strong privacy protections include the Health Information Technology for Economic and Clinical Health (HITECH) Act and the Family Educational Rights and Privacy Act (FERPA). Many statutory standards predate modern AI and do not address the unique security and privacy challenges of adaptive algorithms, cloud-based model updates, or the extensive data sharing required for AI training and validation. While data sharing and interoperability are key aspects of health care delivery and research, it is vital that data sharing policies are strengthened to encompass the use of AI and the expansion of AI models.

Overall, the exponential growth in AI technology necessitates routine examination of current and emerging models, data management and sharing policies, and HIPAA compliance. Currently, health care organizations routinely sign business associate agreements (BAAs) that may permit extensive data sharing that creates ethical transparency gaps in data governance. Despite protective provisions under the HIPAA Privacy Rule and Security Rule, patients and clinicians may lack information and sufficient control over data sharing in a markedly different care environment shaped by AI. Transparent disclosure is needed for BAAs and data sharing arrangements that govern how patient information is used to train, validate, and improve AI systems. Increased standards under the HIPAA Privacy Rule could require transparent disclosure of BAAs and other data sharing arrangements to clinicians and individuals. Additionally, the HIPAA Security Rule could be updated to incorporate contemporary cybersecurity frameworks for electronic protected health information (ePHI) such as standards from the National Institute of Standards & Technology (NIST) and International Organization for Standardization (ISO), for example, ensuring health care entities have clear, robust guidance for protecting patient data in AI-driven environments. It is important to include nurses (including nurse scientists and informaticists) in governance committees, procurement decisions, post-market monitoring, and evaluation processes to guide responsible AI implementation and navigate these issues.

Question 9: What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Nurses and other clinicians have a critical role to play in promoting AI literacy and fostering understanding for patients and clinicians on the use of AI in health care. When surveyed, many people in the United States note some level of concern about use of AI in daily life, and awareness of common uses of AI can vary widely.⁵ As patient educators and advocates, nurses and other clinicians can support patient populations by providing information and protecting their data.

As AI grows in patient care, nurses and other health care professionals may have concerns regarding AI replacing their roles as well as how AI technology could threaten the quality and safety of care for patients and communities.⁶ At the same time, many nurses recognize AI's current ability and future potential to alleviate workload stress points while innovating care. They express a desire for increased AI support in their work.⁷ To guide AI development, implementation, and evaluation, it is critical to maintain an overarching focus on the specific function AI holds in care delivery. While AI can create efficiencies that allow clinicians to better focus on and enhance patient care, AI cannot replace their critical thinking and clinical judgment,⁸ nor can it replace the human connection and care that are central to the clinician-patient relationship.⁹ From an ethical perspective, with AI's integration into the health care system, administration, operations, and clinical care, a triad of the patient, clinician, and AI must be maintained where one entity cannot engage without the others.¹⁰

Question 10: Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

There is a need for comprehensive AI fluency research to guide the education of current and future health care professionals and promote ethical and responsible use of AI in individual care. Health care professionals and populations who use AI tools may currently lack an understanding of their underlying algorithms, data sources, potential biases, or security vulnerabilities. Unlike traditional software with transparent logic paths, AI models often cannot explain how they reach conclusions. AI algorithms (particularly using machine learning and deep learning) can lack transparency and explainability, creating a "black box" issue.¹¹ Therefore, incorrect outputs may not be easily discoverable. Similarly, large language models (LLMs) have the potential to "hallucinate," or generate false information as an output,¹² and algorithms can drift and degrade over time as patterns change.¹³ The AI fluency gap is a pressing and current issue, as consumers are already integrating AI into health decisions outside of clinical settings. For example, *ChatGPT Health* was announced in January 2026 as a platform for people to connect their medical records and wellness apps with the AI chatbot technology.¹⁴ Therefore, AI fluency is important to address not only as AI is increasingly used by clinicians in care, but also as people increasingly use AI for health purposes in everyday life.

If errors occur, this can threaten misdiagnoses and pose ethical issues for individual autonomy and informed consent, creating significant risks for both individuals and clinicians.¹¹ For this reason, education is critical to ensure that the health care workforce will attain the fundamental AI literacy necessary for safe and effective development, implementation, and evaluation of AI.¹⁵ AI literacy is important as a longitudinal, lifetime professional competency rather than a discrete training requirement. It is also important to note that AI fluency research is needed in parallel with education updates. Educational reforms cannot wait for research, though research remains critical to continue to guide these programs into the future.

Comprehensive AI fluency research, with thorough investigation of competency frameworks, educational strategies, workforce readiness, and evaluation of learning outcomes, is needed to guide current and future health care professionals. Education is also critical for patient populations to attain the

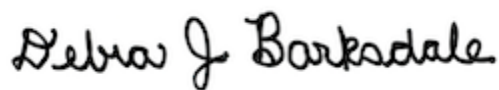
fundamental AI literacy necessary for safe and responsible use of AI. Above all, the triad structure of health care in the new era should be maintained—patient, clinician, and AI—both human wisdom and sourced intelligence.

AI is transforming nursing and other clinical practice by enabling tools such as virtual assistants and chatbots to handle routine tasks, freeing clinicians to focus on complex care. Workflows, task routing, auto-scheduling, and reminders to patients can all be improved through AI-enabled technology. Predictive analytics can help personalize interventions, allowing clinicians to anticipate and address patient needs more effectively and efficiently. Implementation of technologies will alter clinician and patient interactions and the delivery of care more broadly, which necessitates research to ensure safe and effective integration. AI-driven ambient documentation systems, for example, which involve varying types of AI scribes that can generate notes for clinicians attending to patients, are already rapidly being deployed across health care settings. Recent evidence highlights the need for comprehensive validation frameworks for this technology as well as research on its impact on documentation quality, clinical decision-making, and interprofessional communication.¹⁶ This validation needs to include quality assessments from clinicians.

AI also has the potential to enhance traditional models of care with new layers of intelligence and flexibility as well as expand care beyond traditional settings to novel settings, including home-based environments. AI tools are advancing faster than care models can absorb, which is an area to explore through research. Research should also explore how to integrate AI without increasing burden and burnout, including examining the cognitive load for clinicians. Research-backed tools that alert nurses when an algorithm's predictive power has "drifted" and is no longer reliable are also needed. All AI use cases merit careful, ongoing study. Furthermore, research must consider whether data variables used in AI tools appropriately capture important factors such as housing stability, income, geography, nutrition, and access to care. Examining AI's impact for both individuals and clinicians in rural, medically underserved, and resource-limited communities is essential to ensure effective deployment.

The Academy thanks HHS for the opportunity to provide comments on accelerating the adoption and use of AI as part of clinical care. We stand ready to work with you to provide expertise and shape policy solutions that champion health and well-being, ensuring healthy lives for all people. If we can be of any assistance to you or your staff, please do not hesitate to contact the Academy's Chief Policy Officer, Christine Murphy, at cmurphy@aannet.org or 202-777-1170.

Sincerely,



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- ¹ Department of Health and Human Services. (2025, December 23). Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care. RIN 0955-AA13; 90 FR 60108.
- ² APRNs include certified nurse-midwives (CNMs), certified registered nurse anesthetists (CRNAs), clinical nurse specialists (CNSs) and nurse practitioners (NPs).
- ³ American Academy of Nursing. (2025, September 11). *Comments RE: CMS-1832-P: Medicare and Medicaid Programs; CY 2026 Payment Policies Under the Physician Fee Schedule and Other Changes to Part B Payment and Coverage Policies; Medicare Shared Savings Program Requirements; and Medicare Prescription Drug Inflation Rebate Program*. https://cdn.ymaws.com/aannet.org/resource/resmgr/policydocuments/2025policyactions/AAN_CMS_PFS_Comments.pdf
- ⁴ National Academies of Sciences, Engineering, and Medicine; National Academy of Medicine; Committee on the Future of Nursing 2020–2030, Flaubert, J. L., Le Menestrel, S., Williams, D. R., & Wakefield, M. K. (Eds.). (2021). *The Future of Nursing 2020-2030: Charting a Path to Achieve Health Equity*. National Academies Press (US). <https://doi.org/10.17226/25982>
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- ⁶ Simpson, R. L. (2024). How nurses can conquer their fear of AI. *American Nurse Journal*, 19(1), 68. <https://doi.org/10.51256/anj012468>
- ⁷ McKinsey & Company. (2024, October 1). *The pulse of nurses' perspectives on AI in healthcare delivery*. <https://www.mckinsey.com/industries/healthcare/our-insights/the-pulse-of-nurses-perspectives-on-ai-in-healthcare-delivery>
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- ¹² Cross, J. L., Choma, M. A., & Onofrey, J. A. (2024). Bias in medical AI: Implications for clinical decision-making. *PLOS Digital Health*, 3(11), e0000651. <https://doi.org/10.1371/journal.pdig.0000651>
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- ¹⁴ OpenAI. (2026, January 7). *Introducing ChatGPT Health: A dedicated experience in ChatGPT designed for health and wellness*. <https://openai.com/index/introducing-chatgpt-health/>
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